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Correct Answer

Next Question

Practice Exam - 1

Question 44 of 117



An 86-year-old man with a history of nonischemic HFrEF (EF 10-15%) s/p implantation of a dual chamber ICD is admitted to the hospital with acutely decompensated HF. Patient's other known medical history includes osteoarthritis. He lives alone and has not seen a doctor in over a decade. Social history includes remote tobacco use and daily ETOH intake. In the emergency room the patient's vital signs are: heart rate of 104 beats per minute, blood pressure is 92/58 mmHg, and respiratory rate is 24/min. On examination he is ill appearing, sitting up, crackles are noted to mid lung fields bilaterally. Cardiovascular exam reveals a heart rate of 114 beats per minute, regular rhythm with S3 gallop present.

Abdominal exam is notable for a palpable liver. Extremities are cool on exam. Chest Xray is notable for vascular congestion. ECG shows sinus tachycardia, RBBB, QRS 146msec. Labs are significant for sodium of 132 mmol/L, potassium of 4.5mmol/L, chloride 93mmol/L, BUN 9mg/dL, creatinine 0.8 mg/dL, albumin 2.4 g/dL, INR 2.2 sec, hemoglobin 11.6 g/dL, platelets of 89. Patient is given furosemide 80mg IVP with minimal urine output, he becomes more lethargic and is started on dobutamine 5mcg/kg/min. His blood pressure improves to 110/64, HR is 128 bpm, and mental status improves as does urine output and respiratory status.

What is the next best step in patient management?

A. Referral for evaluation for advanced therapies

B. Referral for upgrade to CRT

C. Referral for palliative care

D. Referral for intermittent inotrope infusions

Commentary References

Testing point: The AHFTC Physician should know when to refer patients with advanced HF to palliative care.

Correct Answer: C. Referral for palliative care.

Rationale: This elderly patient who lives alone and has poor self-care and daily alcohol intake, with evidence of malnutrition, presents with evidence of multisystem organ dysfunction, particularly liver dysfunction. Given his age, social circumstances, malnutrition and evident liver dysfunction, referral to palliative care is the most appropriate therapeutic choice.

Answer A: Referral for evaluation for advanced therapies. The patient has multiple contraindications to advanced therapies.

Answer B: Referral for upgrade to CRT. CRT upgrade is not recommended in patients with Stage D heart failure or advanced heart failure.

Answer D: Referral for intermittent dobutamine infusion. The patient has advanced heart failure without significant therapeutic options. While inotrope infusion may be used for symptom relief, this should be provided via continuous infusion. There is no role for intermittent inotrope infusions which increases risk of morbidity and mortality and provide only brief symptom relief.

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